

7 KEY PARTNERS

- Hospitals: International Medical Center, Dr. Soliman Fakeeh Hospital and United Doctors Hospital — pilot sites and validation data.
- Effat University — supervision, research infrastructure and lab access.
- Ministry of Health and the Vision 2030 digital-health programmes — reach and integration.
- Data holders: the NICHHD DASH repository and the RAHMA study team.
- SFDA — the regulatory pathway. StartSmart Universities Challenge — funding, mentorship and visibility.



5 KEY ACTIVITIES

- Build and maintain the self-adaptive framework: Woman Schema, Data Profiler, model-selection policy.
- Obtain and govern clinical data — data use agreements, ethics approval, secure handling.
- Clinical validation with partner hospitals, reporting detection rates together with their false-positive rates.
- Regulatory work: the SFDA Software as a Medical Device pathway, and PDPL compliance.
- Bilingual product design, clinic onboarding and training.

6 KEY RESOURCES

- Data — the hardest asset to obtain and the real moat: the NICHHD DASH cohorts (HRA, LRA, nuMoM2b) and Saudi data (RAHMA).
- The framework itself: the Woman Schema and the Data Profiler are the intellectual property.
- People: the founding team, Dr. Naila Marir (academic supervisor) and Dr. Rania (clinical collaborator).
- Secure cloud infrastructure that complies with the Personal Data Protection Law.
- An Arabic-first health brand that women trust.

1 VALUE PROPOSITIONS

- Know your risk early, in your own language. BADIRA returns three signals together and never a bare number.
- Honest about uncertainty. When the data is not enough, BADIRA says so and asks for more instead of guessing.
- A bridge to the doctor, not a replacement. It produces a summary the woman can share at her next visit.
- For clinics: a triage layer that flags high-risk pregnancies inside the 11–14 week window, while prevention still works.
- What no competitor has: BADIRA adapts to the data. It profiles the dataset first and then chooses the model — so it works on Saudi data, not only on data borrowed from abroad.

EVERY RESULT CARRIES THREE THINGS TOGETHER

Risk — how high it is
Reliability — how sure we are
Time — by when it matters

“Know early. Act in time.”

4 CUSTOMER RELATIONSHIPS

- Get: free for the woman and introduced by her own clinic — no cost barrier at the moment she needs it.
- Keep: a companion for the whole pregnancy, not a one-off test. The risk is recalculated as new information arrives.
- Grow: the summary she brings to her doctor turns every user into a route into a new clinic.
- Trust is the retention strategy: every prediction carries a confidence score, and BADIRA never claims a diagnosis.

3 CHANNELS

- App Store and Google Play — bilingual Arabic/English from day one.
- Referral at the clinic: the doctor introduces BADIRA at the first antenatal visit.
- Integration with Ministry of Health platforms (Sehhaty) and hospital records.
- Awareness campaigns with health influencers — necessary, because only about 27% of Saudi women know what preeclampsia is.

2 CUSTOMER SEGMENTS

- User: pregnant women in Saudi Arabia and the Arab world — first pregnancy, known risk factors, or far from specialist care.
- Paying customer: private clinics, maternity hospitals and antenatal units.
- Institutional: health insurers, and Ministry of Health digital-health programmes under Vision 2030.
- A multi-sided market: the woman uses BADIRA, the clinic pays for it, the insurer benefits from it.
- Entry focus: private maternity clinics in Jeddah and Riyadh first, then Ministry of Health primary-care centres.

9 COST STRUCTURE

- AI development and maintenance — the largest ongoing cost.
- Data acquisition, secure hosting, and privacy and security compliance.
- Clinical advisory team and validation studies.
- SFDA regulatory submission and certification.
- Marketing, clinic partnerships and onboarding.
- Through Senior Project I these are carried by the university; they become real costs at commercialisation.

8 REVENUE STREAMS

- Clinic subscription (primary): an annual licence per clinic, or a per-patient screening fee.
- Insurer partnerships, paid against prevented severe cases — one preeclampsia admission costs far more than screening a whole clinic.
- Freemium for women: the core risk signal free; the full pregnancy timeline and doctor-ready reports paid.
- Grants: Ministry of Health digital-health funding, and research and innovation grants.
- Free during the pilot. Revenue begins only after clinical validation — we do not sell an unvalidated prediction.